JONA
Volume 55, Number 8, pp 457-464
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THE JOURNAL OF NURSING ADMINISTRATION

The Influence of Pathway to Excellence® Designation on Patient Satisfaction

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OBJECTIVE: This study investigated the association of Pathway to Excellence® (Pathway) status with patient satisfaction.
BACKGROUND: The Pathway program recognizes hospitals committed to fostering optimal practice environments, yet research on its impact on patient satisfaction is limited.
METHODS: This cross-sectional study linked 3 datasets: the Hospital Consumer Assessment of Healthcare Providers and Systems, the American Nurses Credentialing Center® Pathway and Magnet® organizations, and the American Hospital Association Annual Survey. The primary analysis included 2721 non-Magnet hospitals (2588 without Pathway status, 133 with Pathway status). The secondary analysis included 606 hospitals (473 Magnet, 133 Pathway). Outcomes included hospital rating and willingness to recommend the hospital. Propensity score–weighted simple linear regression estimated outcome differences.
RESULTS: Compared with hospitals without Pathway or Magnet status, Pathway hospitals had significantly higher patient ratings and recommendations. No significant differences were found between Pathway and Magnet hospitals for either patient satisfaction outcome.
CONCLUSIONS: Pathway designation and thus the journey to achieve this designation may be an effective strategy for improving patient satisfaction.

Patient satisfaction remains a critical indicator of hospital performance despite ongoing criticism regarding the validity of satisfaction measures and their potential misalignment with clinical outcomes.[1,2] It is frequently associated with broader care quality measures, clinical outcomes, and hospital reputation.[3,4] The Hospital Consumer Assessment of Healthcare Providers and Systems (HCAHPS) survey, developed by the Centers for Medicare & Medicaid Services (CMS), is the primary tool for assessing patient experience and satisfaction in US hospitals. This standardized survey enables national comparisons of patient experiences across hospitals.[5] Additionally, HCAHPS scores are tied to financial incentives, as part of hospital reimbursement under the Value-Based Purchasing Program is based on these scores.[6] This connection underscores the significance of patient satisfaction in the healthcare landscape and its impact on hospital financial performance.

Among the various items covered in the HCAHPS survey, 2 key global measures are frequently used to evaluate overall patient satisfaction with hospital care: 1) the overall hospital rating; and 2) patients' willingness to recommend the hospital to others.[7,8] These measures capture patients' broad perceptions of the care received during their hospital stay and their likelihood of endorsing the hospital to family and friends, making them critical metrics for hospitals striving to enhance patient-centered care. Hospitals often showcase these scores on their websites to enhance transparency and attract potential patients by demonstrating their commitment to high-quality care.[9,10]

HCAHPS measures are linked to various structural and operational characteristics of hospitals.[11] Positive predictors of HCAHPS scores include nonprofit status,

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Funding/support: no funding to report.
The authors declare no conflicts of interest.
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Supplemental digital content is available for this article. Direct URL citations appear in the printed text and are provided in the HTML and PDF versions of this article on the journal's Web site (www.jonajournal.com).
DOI: 10.1097/NNA.0000000000001610

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specialty focus, and the use of electronic health records. Work environment attributes, including a compassionate organizational culture, clearly defined nursing standards for high-quality care, cultural competency, and positive safety culture perceptions, are also associated with higher HCAHPS scores.[11] Other positive predictors are higher surgical volumes and favorable nurse-to-patient ratios. Conversely, negative predictors include safety net facilities, longer nursing shifts, and a higher proportion of Medicaid patients. Some predictors, including teaching status and hospital size, show inconsistent relationships, resulting in mixed findings regarding their impact on HCAHPS scores.[11]

Nursing-specific recognition programs are also associated with HCAHPS measures. Studies show that Magnet® hospitals outperform non-Magnet hospitals on HCAHPS.[12-14] However, limited research exists on the relationship between Pathway to Excellence® (Pathway) status and patient satisfaction. The Pathway program recognizes healthcare organizations committed to creating optimal practice environments for nurses, reflecting an organization's adherence to evidence-based practices that foster a supportive and empowering environment for nursing staff.[15-17] The program focuses on 6 key areas: 1) *shared decision-making* to involve direct care nurses in collaboration and decision-making; 2) *leadership* that promotes shared governance and prioritizes development and retention; 3) *safety*, ensuring a respectful, violence-free environment for both nurses and patients; 4) *quality*, emphasizing person-centered care and continuous improvement; 5) *well-being*, supporting nurse recognition and health resources; and 6) *professional development*, providing access to mentoring and lifelong learning opportunities.[15-17]

The Pathway and Magnet programs both promote supportive work environments and evidence-based nursing but differ in scope. Magnet standards are more comprehensive, focusing on leadership, structural empowerment, and system-wide innovation, whereas Pathway focuses on fostering positive work environments through shared decision-making and professional development. Although Pathway standards are similar to Magnet, they are less rigorous in terms of empirical outcomes, data collection, and benchmarking, making them more accessible to smaller facilities.[18] Along with comparisons to hospitals without Pathway and Magnet status, understanding how Pathway hospitals perform in terms of patient satisfaction compared with Magnet hospitals can help healthcare organizations choose recognition programs that best align with their size, resources, and goals for patient-centered care. This study investigated patient satisfaction between Pathway and hospitals without Pathway and Magnet status, as well as between Pathway and Magnet hospitals.

Methods

This study used a cross-sectional study design. Given the absence of patient-level data or protected health information and the exclusive use of deidentified, hospital-level data, this study received an exemption from institutional review board review at the University of Pennsylvania.

Data Sources

Researchers linked data from 3 sources: 1) the HCAHPS; 2) the American Nurses Credentialing Center® (ANCC) Pathway and Magnet organizations; and 3) the American Hospital Association (AHA) Annual Survey. HCAHPS is a standardized survey collecting patient feedback on hospital care through random sampling of discharged patients. Responses are collected post-discharge via mail, telephone, mail with telephone follow-up, or interactive voice recognition, covering various aspects of the hospital experience. These results are then aggregated and publicly reported to compare hospital performance.[5] Data on hospitals with Pathway and Magnet status are regularly updated by the ANCC.[19,20] The AHA survey gathers detailed information on hospital operations, including staffing and services, through annual questionnaires sent to hospitals across the United States.[21] HCAHPS and ANCC data are publicly available, whereas AHA data require purchasing a license.

Researchers analyzed hospital-level data from the 2023 HCAHPS survey. Data on Pathway- and Magnet-designated organizations were used to categorize hospitals in the 2023 data into 3 groups: 1) hospitals without Pathway or Magnet status; 2) Magnet-designated hospitals; and 3) Pathway-designated hospitals. The 2022 AHA Annual Survey was used to account for hospital characteristics.

Study Sample

To evaluate the relationship between Pathway status and HCAHPS scores, researchers established the following inclusion criteria: 1) hospitals situated in US states; 2) participation in HCAHPS in 2023 with non-missing values; and 3) participation in the 2022 AHA Annual Survey. Researchers identified 4791 hospitals that participated in the HCAHPS surveys in 2023. Initially, 12 hospitals located in US territories were excluded to avoid introducing variability stemming from different healthcare policies and regulations, which could distort the results and hinder comparability with hospitals on the US mainland. Researchers then excluded 1565 hospitals due to missing HCAHPS values, as mandated by CMS's policy, excluding hospitals with fewer than 100 completed HCAHPS patient surveys within a specified 4-quarter period. Additionally, 20

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hospitals were removed that did not participate in the 2022 AHA Annual Survey.

The remaining 3194 hospitals were categorized based on their Magnet and Pathway status. The primary analysis excluded 473 Magnet hospitals to focus on hospitals without Magnet designation, as Magnet hospitals often have higher patient satisfaction and better outcomes,[22] which could confound the results. Thus, the primary analysis included 2721 non-Magnet hospitals: 1) 133 that held Pathway status in 2023; and 2) 2588 that did not have this designation in 2023. The secondary analysis included 606 hospitals to compare 133 Pathway hospitals with 473 Magnet hospitals.

Study Variables

Outcome: Patient Satisfaction on Hospital Care

Patient satisfaction in hospital care was assessed using 2 measures: 1) the overall hospital rating; and 2) patients' willingness of recommending the hospital. Hospital ratings were evaluated using the HCAHPS survey question: "Using any number from 0 to 10, where 0 is the worst hospital possible and 10 is the best hospital possible, what number would you use to rate this hospital during your stay?" Respondents had 11 options to choose from, ranging from 0 to 10. Patient recommendations were measured by the HCAHPS survey question: "Would you recommend this hospital to your friends and family?" Respondents selected from 4 options: "definitely no," "probably no," "probably yes," and "definitely yes," scored as 1, 2, 3, and 4, respectively.[23] CMS determines and publishes the linear mean scores for hospitals regarding these outcomes through a multistep approach: 1) individual responses are assigned numeric values and averaged for each hospital over a 12-month period; 2) these average scores are then adjusted quarterly, taking into account patient demographics and survey mode; and 3) the adjusted scores are converted to a scale of 0 to 100, with final results rounded to whole numbers. Details about the score calculation, including adjustments for patient mix and survey mode, are described elsewhere.[24] The reported scores reflect a rolling average over 4 quarters, weighted according to the number of eligible patients at each hospital, with higher scores indicating better ratings and recommendations.

Hospital Characteristics

Researchers included various structural and geographical hospital characteristics in the propensity score (PS) model as these factors might bias the association between Pathway status and patient satisfaction outcomes. These included teaching status, hospital size, technology status, ownership, region, and urban location.

As hospitals' teaching status was associated with patient satisfaction,[11] it was accounted for to ensure a balanced comparison. Teaching status was evaluated using the ratio of resident physicians and fellows to hospital beds, considered more accurate than university affiliations for evaluating teaching activity.[25] Hospitals were categorized based on their trainee-to-bed ratios: those without any trainees (nonteaching hospitals), those with a trainee-to-bed ratio of 1:4 or lower (minor teaching hospitals), and those with a ratio exceeding 1:4 (major teaching hospitals). For technology status, hospitals equipped for open heart surgery and/or major transplants were classified as high-technology hospitals, compared with those without such facilities. Hospital size, linked to patient satisfaction,[26] was categorized as small (up to 100 beds), medium (101-250 beds), and large (>250 beds).[27] Ownership types included not-for-profit, for-profit, and federal. Regional classifications were based on US Census data, including the Midwest, Northeast, South, and West.[28] Finally, urban locations were categorized using core-based statistical area types, which were merged into metropolitan (including rural and micropolitan areas) and nonmetropolitan categories.

Statistical Analysis

All data analyses were performed using R version 4.4.1 (R Foundation for Statistical Computing, Vienna, Austria). Researchers began with bivariate analyses to assess whether patient satisfaction outcomes and hospital characteristics varied by Pathway status. Next, researchers fitted a PS model in which Pathway status was predicted by hospital characteristics (teaching status, hospital size, technology status, urban location, region, and ownership) to reduce bias in estimating the association between Pathway status and outcomes.[29] The PS reflects the conditional probability of a hospital having Pathway status compared with one without it, given the observed confounding factors. Researchers utilized this probability to generate weights[30,31] for hospitals without Pathway status. Hospitals without Pathway status that shared similar characteristics with Pathway hospitals were assigned higher weights (up-weighted), whereas those with differing characteristics received lower weights (down-weighted). For example, because Pathway hospitals tend to be small-sized, metropolitan, nonteaching, and non-profit (Table 1), a non-Pathway hospital with similar characteristics would be up-weighted due to its close alignment. Conversely, a large, for-profit, high-technology hospital in a rural area would be down-weighted due to its differing characteristics, making it less relevant for estimating the association between Pathway status and patient satisfaction outcomes.

Researchers fitted the PS model using the TWANG R package,[32] which utilizes generalized boosted models instead of standard logistic regression.[33] The Kolmogorov-Smirnov (KS) statistic was used to compare

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Table 1. Characteristics Among Pathway, Magnet, and Non-Pathway/Non-Magnet Hospitals

	Total Hospitals (n = 3194)	Pathway Hospitals (n = 133)	Magnet Hospitals (n = 473)	Non-Magnet/Non-Pathway Hospitals (n = 2588)	*P*
Overall hospital rating, mean (SD)	87.4 (4.0)	88.2 (3.2)	88.1 (2.6)	87.2 (4.2)	<0.001
Overall hospital rating, range	65-98	76-96	78-96	65-98	
Patient recommendation, mean (SD)	86.3 (5.1)	87.3 (4.2)	88.1 (3.4)	86.0 (5.3)	<0.001
Patient recommendation, range	58-99	71-95	75-97	58-99	
Hospital size, n (%)					<0.001
Small	1164 (36.4)	53 (39.8)	18 (3.8)	1093 (42.2)	
Medium	1043 (32.7)	46 (34.6)	142 (30.0)	855 (33.0)	
Large	987 (30.9)	34 (25.6)	313 (66.2)	640 (24.7)	
Urban location, n (%)					<0.001
Metropolitan	2359 (73.9)	110 (82.7)	460 (97.3)	1759 (59.1)	
Nonmetropolitan	835 (26.1)	23 (17.3)	13 (2.7)	799 (30.9)	
Region, n (%)					<0.001
Midwest	847 (26.5)	16 (12.0)	127 (26.8)	704 (27.2)	
Northeast	481 (15.1)	16 (12.0)	105 (22.2)	360 (13.9)	
South	1189 (37.2)	85 (63.9)	155 (32.8)	949 (36.7)	
West	677 (21.2)	16 (12.0)	86 (18.2)	575 (22.2)	
Teaching status, n (%)					<0.001
None	1711 (53.6)	74 (55.6)	209 (44.2)	1428 (55.2)	
Minor	1177 (36.9)	41 (30.8)	150 (31.7)	986 (38.1)	
Major	306 (9.6)	18 (13.5)	114 (24.1)	174 (6.7)	
Technology status, n (%)					<0.001
High technology	964 (30.2)	39 (29.3)	305 (64.5)	620 (24.0)	
Low technology	2230 (69.8)	94 (70.7)	168 (35.5)	1938 (76.0)	
Ownership, n (%)					<0.001
Not-for-profit	2490 (78.0)	108 (81.2)	455 (96.2)	1927 (74.5)	
For-profit	563 (17.6)	15 (11.3)	15 (3.2)	533 (20.6)	
Federal	141 (4.4)	10 (7.5)	3 (0.6)	128 (4.9)	

covariate distributions between the 2 groups, with KS values exceeding 0.10 suggesting possible imbalances.[34] Once balance was achieved, researchers estimated the effect of Pathway status by fitting a PS-weighted simple linear regression to assess mean differences in 2 patient satisfaction outcomes.

Results

Pathway Hospitals Versus Hospitals Without Pathway or Magnet Status

Table 1 summarizes hospital characteristics among Pathway, Magnet, and non-Pathway/non-Magnet hospitals. The geographic locations of Pathway hospitals are presented in Supplemental Digital Content 1, http://links.lww.com/JONA/B302. Of the 2721 non-Magnet hospitals, including Pathway and those without Pathway status, 42.1% were small-sized (Table 2). The majority (69.8%) were located in metropolitan areas, and 38% were situated in the South region. More than half of the hospitals (55.2%) were nonteaching institutions. Approximately one-quarter (24.2%) had high technology status, whereas the majority (74.8%) were nonprofit organizations. Unadjusted analyses prior to weighting revealed that the average hospital recommendation score was significantly higher for Pathway hospitals (mean, 87.3 [SD, 4.2]; range, 71-95) compared with those without Pathway or Magnet status (mean, 86.0 [SD, 5.3]; range, 58-99) ($P = 0.006$). Similarly, Pathway hospitals also achieved a higher average hospital rating score (mean, 88.2 [SD, 3.2]; range, 76-96) than those without Pathway or Magnet status (mean, 87.2 [SD, 4.2]; range, 65-98) ($P = 0.007$). Baseline characteristics before and after PS weighting are presented in Table 2. After applying PS weighting, the KS statistics for all confounding factors fell below 0.008, indicating that the weighting effectively balanced the distribution of these confounding factors between Pathway hospitals and hospitals without Pathway or Magnet status.

Table 3 presents mean differences in outcomes between Pathway hospitals and hospitals without Pathway or Magnet status based on PS-weighted simple linear regression analyses. Compared with hospitals without Pathway or Magnet status, Pathway hospitals had significantly higher patient ratings by 0.8 points ($P = 0.012$) and 0.9-point higher patient recommendations ($P = 0.03$).

Pathway Versus Magnet Hospitals

Table 4 summarizes the characteristics of Pathway and Magnet hospitals. Compared with Magnet hospitals, Pathway hospitals were smaller in size (40% vs 4%; $P < 0.001$), located in nonmetropolitan areas (17% vs 3%; $P < 0.001$) and the South region (64% vs 33%; $P < 0.001$), and nonteaching (56% vs 44%; $P = 0.016$); had lower technology status (71% vs 35%;

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Table 2. Baseline Characteristics Before and After Propensity Score Weighting Between Pathway and Non-Pathway Hospitals

		Before Propensity Score Weighting				After Propensity Score Weighting			
	Total Hospitals (n = 2721)	Pathway Group, Mean (SD)	Non-Pathway Group, Mean (SD)	*P*	KS Statistic [a]	Pathway Group, Mean (SD)	Non-Pathway Group, Mean (SD)	*P*	KS Statistic
Hospital size, n (%)				0.862				0.988	
Small	1146 (42.1)	0.40 (0.49)	0.42 (0.49)		0.024	0.40 (0.49)	0.39 (0.49)		0.007
Medium	901 (33.1)	0.35 (0.48)	0.33 (0.47)		0.015	0.35 (0.48)	0.35 (0.48)		0.003
Large	674 (24.8)	0.26 (0.44)	0.25 (0.43)		0.008	0.26 (0.44)	0.26 (0.44)		0.004
Urban location, n (%)				0.001				0.954	
Metropolitan	1899 (69.8)	0.83 (0.38)	0.69 (0.46)		0.136	0.83 (0.38)	0.83 (0.38)		0.002
Nonmetropolitan	822 (30.2)	0.17 (0.38)	0.31 (0.46)		0.136	0.17 (0.38)	0.18 (0.38)		0.002
Region, n (%)				<0.001				1.000	
Midwest	720 (26.5)	0.12 (0.33)	0.27 (0.44)		0.152	0.12 (0.33)	0.12 (0.32)		0.002
Northeast	376 (13.8)	0.12 (0.33)	0.14 (0.35)		0.019	0.12 (0.33)	0.12 (0.32)		0.002
South	1034 (38.0)	0.64 (0.48)	0.37 (0.48)		0.272	0.64 (0.48)	0.64 (0.48)		0.005
West	591 (21.7)	0.12 (0.33)	0.22 (0.42)		0.102	0.12 (0.33)	0.12 (0.33)		0.000
Teaching status, n (%)				0.006				0.992	
None	1502 (55.2)	0.56 (0.50)	0.55 (0.50)		0.005	0.56 (0.50)	0.56 (0.50)		0.001
Minor	1027 (37.7)	0.31 (0.46)	0.38 (0.49)		0.073	0.31 (0.46)	0.31 (0.46)		0.004
Major	192 (7.1)	0.14 (0.34)	0.07 (0.25)		0.068	0.14 (0.34)	0.13 (0.34)		0.003
Technology status, n (%)				0.159				0.846	
High technology	659 (24.2)	0.29 (0.46)	0.24 (0.43)		0.054	0.29 (0.46)	0.29 (0.45)		0.008
Low technology	2062 (75.8)	0.71 (0.46)	0.76 (0.43)		0.054	0.71 (0.46)	0.72 (0.45)		0.008
Ownership, n (%)				0.020				0.999	
Not-for-profit	2035 (74.8)	0.81 (0.39)	0.75 (0.44)		0.067	0.81 (0.39)	0.81 (0.39)		0.001
For-profit	548 (20.1)	0.11 (0.32)	0.21 (0.40)		0.093	0.11 (0.32)	0.11 (0.32)		0.001
Federal	138 (5.1)	0.08 (0.26)	0.05 (0.22)		0.026	0.08 (0.26)	0.08 (0.26)		0.000

[a]KS statistics greater than 0.10 are considered as indications of imbalance.

$P < 0.001$); and were for-profit or federally owned (19% vs 4%; $P < 0.001$). Unadjusted analyses before weighting indicated that the average patient recommendations were significantly higher for Pathway hospitals (mean, 87.3 [SD, 4.2]; range, 71-95) compared with Magnet hospitals (mean, 88.1 [SD, 3.4]; range, 75-97) ($P = 0.017$). However, there was no statistically significant difference in hospital ratings between Pathway (mean, 88.2 [SD, 3.2]; range, 76-96) and Magnet hospitals (mean, 88.1 [SD, 2.6]; range, 78-96) ($P = 0.900$). Baseline characteristics before and after PS weighting are also presented in Table 4. After PS weighting, the KS statistics for all confounding factors fell below 0.04, indicating effective balancing of these factors between Pathway and Magnet hospitals.

Table 5 presents mean differences in outcomes based on PS-weighted simple linear regression analyses. There was no significant difference in hospital ratings ($P = 0.254$) or patient recommendations ($P = 0.099$) between Pathway and Magnet hospitals.

Discussion

These findings support the hypothesis that Pathway hospitals are associated with higher levels of patient satisfaction compared with hospitals without Pathway or Magnet status. Although the differences in HCAHPS scores are statistically significant, the mean difference between Pathway hospitals and those without is relatively small. However, hospitals without Pathway or Magnet

Table 3. Outcome Differences Between Pathway and Non-Pathway Hospitals

	Overall Hospital Rating			Willingness to Recommend the Hospital		
	Before Weighting, Mean (SD)	After Weighting, Mean (SD)	*P*	Before Weighting, Mean (SD)	After Weighting, Mean (SD)	*P*
Pathway status			0.012[a]			0.03[a]
No	87.2 (4.2)	87.4 (3.8)		86.0 (5.3)	86.4 (4.9)	
Yes	88.2 (3.2)	88.2 (3.2)		87.3 (4.2)	87.3 (4.2)	

[a]$P < 0.05$.

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Table 4. Baseline Characteristics Before and After Propensity Score Weighting Between Pathway and Magnet Hospitals

		Before Propensity Score Weighting				After Propensity Score Weighting			
	Total Hospitals (n = 606)	Pathway Group, Mean (SD)	Magnet Group, Mean (SD)	*P*	KS Statistic [a]	Pathway Group, Mean (SD)	Magnet Group, Mean (SD)	*P*	KS Statistic
Hospital size, n (%)				<0.001				0.854	
Small	71 (11.7)	0.40 (0.49)	0.04 (0.19)		0.360	0.40 (0.49)	0.37 (0.48)		0.032
Medium	188 (31.0)	0.35 (0.48)	0.30 (0.46)		0.046	0.35 (0.48)	0.37 (0.48)		0.024
Large	347 (57.3)	0.26 (0.44)	0.66 (0.47)		0.406	0.26 (0.44)	0.26 (0.44)		0.008
Urban location, n (%)				<0.001				0.851	
Metropolitan	570 (94.1)	0.83 (0.38)	0.97 (0.16)		0.145	0.83 (0.38)	0.84 (0.37)		0.014
Nonmetropolitan	36 (5.9)	0.17 (0.38)	0.03 (0.16)		0.145	0.17 (0.38)	0.16 (0.37)		0.014
Region, n (%)				<0.001				0.844	
Midwest	143 (23.6)	0.12 (0.33)	0.27 (0.44)		0.148	0.12 (0.33)	0.12 (0.33)		0.000
Northeast	121 (20.0)	0.12 (0.33)	0.22 (0.42)		0.102	0.12 (0.33)	0.14 (0.35)		0.021
South	240 (39.6)	0.64 (0.48)	0.33 (0.47)		0.311	0.64 (0.48)	0.65 (0.48)		0.011
West	102 (16.8)	0.12 (0.33)	0.18 (0.39)		0.062	0.12 (0.33)	0.09 (0.29)		0.031
Teaching status, n (%)				0.016				0.939	
None	283 (46.7)	0.56 (0.50)	0.44 (0.50)		0.115	0.56 (0.50)	0.57 (0.50)		0.015
Minor	191 (31.5)	0.31 (0.46)	0.32 (0.47)		0.009	0.31 (0.46)	0.31 (0.46)		0.001
Major	132 (21.8)	0.14 (0.34)	0.24 (0.43)		0.106	0.14 (0.34)	0.12 (0.32)		0.017
Technology status, n (%)				<0.001				0.702	
High technology	262 (43.2)	0.29 (0.46)	0.65 (0.48)		0.352	0.29 (0.46)	0.32 (0.47)		0.022
Low technology	344 (56.8)	0.71 (0.46)	0.35 (0.48)		0.352	0.71 (0.46)	0.69 (0.47)		0.022
Ownership, n (%)				<0.001				0.991	
Not-for-profit	563 (93.1)	0.81 (0.39)	0.96 (0.19)		0.150	0.81 (0.39)	0.80 (0.40)		0.008
For-profit	30 (5.0)	0.11 (0.32)	0.03 (0.18)		0.081	0.11 (0.32)	0.12 (0.32)		0.005
Federal	13 (2.1)	0.08 (0.26)	0.01 (0.08)		0.069	0.08 (0.26)	0.08 (0.27)		0.003

[a]KS statistics greater than 0.10 are considered as indications of imbalance.

status exhibited a much wider range of scores, with some achieving considerably lower minimum scores. This variability could result in a higher proportion of Pathway hospitals qualifying for Medicare payment incentives, underscoring the importance of pursuing Pathway status to enhance patient satisfaction and secure financial benefits. Therefore, hospitals seeking to improve patient satisfaction should consider pursuing this designation.

Several factors may explain the improved patient satisfaction seen in Pathway hospitals. First, the Pathway program prioritizes an optimal practice environment for nurses, supporting their ability to work effectively while feeling empowered and engaged.[15] Studies have consistently shown a positive relationship between a supportive work environment and higher HCAHPS scores.[35,36] The program also focuses on nurses' well-being, recognizing the importance of a healthy workforce for delivering high-quality care.[15] By reducing burnout and enhancing job satisfaction,[37] the Pathway program may help nurses provide more compassionate and patient-centered care, contributing to better patient experiences.[38] Future research should explore how Pathway status influences nurse job outcomes to further understand these dynamics.

The commitment to safety and quality care in Pathway hospitals may enhance patient satisfaction.[15] Previous research has shown a significant relationship between hospital safety and patient satisfaction.[39] The Pathway program fosters a secure, respectful, and supportive environment,[15-17] which is targeted to protect patients and build their trust in the healthcare team. Additionally, the program emphasizes high-quality,

Table 5. Outcome Differences Between Pathway and Magnet Hospitals

	Overall Hospital Rating			Willingness to Recommend the Hospital		
	Before Weighting, Mean (SD)	After Weighting, Mean (SD)	*P*	Before Weighting, Mean (SD)	After Weighting, Mean (SD)	*P*
			0.254			0.099
Magnet hospitals	88.1 (2.6)	88.7 (2.9)		88.1 (3.4)	88.3 (4.0)	
Pathway hospitals	88.2 (3.2)	88.2 (3.2)		87.3 (4.2)	87.3 (4.2)	

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individualized care through evidence-based practices and continuous improvement strategies aligned with modern clinical guidelines and person- and family-centered care principles.[15] This combination of safety and dedication to quality care significantly contributes to positive patient perceptions during their hospital stay. Future research should examine the program's impact on other patient outcomes, including health improvement and recovery rates, to gain a more comprehensive understanding of the benefits associated with Pathway status.

The secondary analysis results indicate no difference in patient satisfaction outcomes, as measured by the national standard patient experience measure, between Pathway and Magnet hospitals. This suggests that both programs may be effective in achieving patient satisfaction. However, this finding is specific to patient satisfaction, and further research is necessary to explore differences in other important outcomes, such as clinical and patient safety outcomes, as well as nurse outcomes. The absence of a significant difference in patient satisfaction highlights the need to evaluate the cost-benefit tradeoffs of the major differences in requirements between the 2 programs. Specifically, future research should assess whether the intensive and costly data collection and benchmarking requirements for Magnet hospitals, including demonstrating unit-level outperformance of a patient experience benchmark from an ANCC-approved vendor's national database, provide sufficient added value compared with the Pathway program. Future research should also investigate specific mechanisms through which nursing excellence, including leadership, structural empowerment, and professional development, directly affects various dimensions of the patient experience.

The strengths of this study include the use of PS weighting to address potential confounding factors and its focus on a largely unexplored area using a national sample. However, several limitations should be acknowledged. First, the cross-sectional design limits the ability to establish causality between Pathway status and patient satisfaction, as the temporal order of variables remains unclear. Longitudinal studies would determine whether Pathway designation directly enhances patient satisfaction. Second, despite using PS weighting, unmeasured confounding, including hospital leadership or nurse staffing ratios, may still exist. Finally, despite the international reach of the Pathway program, this study focused only on US hospitals, limiting the generalizability of the findings to healthcare systems in other countries.

Conclusions

This study highlights a significant association between Pathway status and patient satisfaction. The findings suggest that obtaining Pathway designation may be an effective strategy for hospitals aiming to improve patient satisfaction. No significant difference in patient satisfaction outcomes between Pathway and Magnet hospitals suggests that both programs may be effective in this regard.

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